Advanced ALK-positive NSCLC after crizotinib Adults with ALK-positive advanced non-small-cell lung cancer previously treated with crizotinib: NCCP 00340a. This suite does not turn the broader first-line licence into an NCCP indication or funding approval. Confirm your treatment setting with the team.

NCCP 00340 · VERSION 4

Ceritinib monotherapy

The usual starting dose is 450 mg once daily with food, every day without a planned rest week. Follow your own prescription. Treatment continues while it helps and side effects remain acceptable; there is no fixed course length.

Discussion aid: review with the patient and complete the approved local consent process.

Purpose and plan

☐ Exact indication, treatment aim and uncertainty explained

☐ Expected duration and review / switch points discussed

☐ Alternatives and the option not to proceed discussed

☐ Patient priorities, capacity and support needs explored

Practical safety

☐ Medicine, route, schedule and missed-dose instructions

☐ Food, interactions and safe handling reviewed

☐ Pregnancy, contraception / fertility and breastfeeding

☐ Monitoring, symptom reporting and contact plan

Selected material risks

☐ Nausea, vomiting and diarrhoea

☐ Lung inflammation

☐ High blood glucose

☐ Poor appetite, fatigue and weight loss

☐ Liver injury

☐ Heart rhythm changes and slow pulse

☐ Pancreatic inflammation

☐ Vision and kidney-test changes

Discuss the post-crizotinib setting, aim and uncertainty, alternatives including not proceeding, material risks, daily treatment and food burden, interactions, pregnancy/fertility/feeding, monitoring and your right to reconsider. This aid is not treatment authorisation.

Individual benefit, alternatives and concerns

Agreed checks and next review

Before starting: ALK test, blood count, kidney and liver tests, pancreatic enzymes, fasting glucose, ECG and symptom/medicine review. Blood count, kidneys and fasting glucose every 4 weeks; liver tests every 2 weeks for the first 3 months, then monthly. ECG after 1 month, then as needed. Extra heart, electrolyte, glucose or pancreatic tests may be needed for risks, symptoms or abnormal results; scans follow the consultant plan.

Clinician / date: _____ Patient discussion documented: _____

Consent is an ongoing conversation and may be reconsidered. This aid is not treatment authorisation.